

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Vu Duc Nghia	MRN	MRN-0030
DOB	1998-02-02	Gender	Male
Encounter Date	2024-05-25	Department	Cardiology
Attending Provider	BS. Tran Van Minh	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Vu Duc Nghia is a 28-year-old patient in Cardiology presenting for follow-up of dyspnea on exertion and lower-extremity edema. PMH includes AFib, HTN, dyslipidemia, possible CKD stage 3. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include apixaban 5mg BID; metoprolol succinate 50mg daily; lisinopril 10mg daily; furosemide 20mg daily. Allergies: sulfonamide allergy - hives.

Objective

Vitals: BP 140/70 mmHg, HR 92 bpm, RR 20/min, Temp 36.6 C, SpO2 94% RA, weight 85 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-04-30	Creatinine	1.63	mg/dL	0.7-1.3	High
2026-04-30	eGFR	54.0	mL/min/1.73m2	>=60	Low
2026-04-30	Glucose	137.0	mg/dL	70-99	High
2026-04-30	HbA1c	7.7	%	<5.7	High
2026-04-30	BNP	554.0	pg/mL	<100	High
2026-04-30	AST	33.0	U/L	10-40	Normal
2026-04-30	ALT	41.0	U/L	7-56	Normal
2026-04-30	Potassium	4.3	mmol/L	3.5-5.0	Normal
2026-04-30	Hemoglobin	13.6	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: acute on chronic heart failure with preserved ejection fraction, atrial fibrillation, hypertension. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Continue telemetry if inpatient. Monitor BNP, Cr, eGFR, K, and daily weight. Adjust diuretic dose based on BP and renal function. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. acute on chronic heart failure with preserved ejection fraction, atrial fibrillation, hypertension. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Cardiology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.